

NAUSEA/VOMITING

- Headache →30
- Chest pain →37
- If patient has watery diarrhoea (with or without vomiting) and has been in cholera outbreak area in past 5 days, **cholera** likely →47
- Neck stiffness, drowsy/confused or purple/red rash: **meningitis** likely

Manage and refer urgently:

- If BP < 90/60 or **pancreatitis** likely, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- If **meningitis** likely, give **ceftriaxone** 2g IV²/IM. Avoid injecting > 1g IM at one injection site.
- If pain severe, give **morphine** 10mg IM or diluted **morphine**³ 3-10mg slow IV: start with 3mL IV over 3 minutes. If needed, give another 1mL/minute until pain improved, up to 10mL. Stop if BP drops < 90/60.
- If glucose < 3 or ≥ 11.1 ⇨ 17 or if diabetes and glucose < 4 ⇨ 130.

Give urgent attention to the patient with nausea/vomiting and any of:

- Guarding, rigidity or rebound tenderness: **peritonitis** likely
- Tender in right lower abdomen: **appendicitis** likely
- Sudden severe upper abdominal pain spreading to back: **pancreatitis** likely
- BP < 90/60
- Vomiting blood
- Jaundice
- Abdominal pain/distention and no stools or flatus/wind
- Drowsy/confused/rapid deep breathing
- If sudden nausea/vomiting and any of: generalised itch/rash, face/tongue swelling, wheeze, difficulty breathing, BP < 90/60, dizziness/collapse or exposure to possible allergen¹, check for anaphylaxis ⇨ 20.

Approach to the patient with nausea/vomiting not needing urgent attention

- If thirst, dry mouth, poor skin turgor or pulse ≥ 100, **dehydration** likely, give single dose **metoclopramide** 10mg orally/IM/IV. Then give **oral rehydration solution** and observe: encourage small frequent sips. Aim for 1-2L in first 2 hours. If vomits, wait 10 minutes and try again more slowly.
- If unable to drink or no better after 2 hours, give **sodium chloride 0.9%** 500mL IV over 30 minutes and refer.
- Exclude pregnancy ⇨ 157. If pregnant, reassure that nausea/vomiting is common in first trimester. Encourage to eat smaller meals more frequently and drink fluids regularly.
- If associated dizziness ⇨ 29.
- Review medication: NSAIDs (e.g. ibuprofen), metformin, contraceptives, hormone therapy, chemotherapy and morphine can cause nausea/vomiting. Discuss with doctor. If on DS-TB medication ⇨ 96, RR-TB medication ⇨ 104 or ART ⇨ 116.
- Screen for alcohol/drug use: in the past year, has patient: 1) drunk ≥ 4 drinks⁴/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ⇨ 142.

Is there recent onset vomiting with cramping abdominal pain, diarrhoea, loss of appetite, body pain or fever?

Yes

Gastroenteritis likely

- If nausea/vomiting, give **metoclopramide** 10mg 8 hourly as needed for up to 5 days.
- Give **oral rehydration solution**.
- If diarrhoea, give **loperamide** 4mg initially, then 2mg after each loose stool if needed, up to 12mg/day.
- If abdominal cramps are distressing, give **hyoscine butylbromide** 10mg 6 hourly for up to 3 days if needed.
- Advise patient to drink lots of fluids, eat small frequent meals as able and avoid fatty food.
- Advise patient to return if symptoms worsen, vomiting > 3 days or not tolerating oral fluids.

No

Does patient have epigastric pain which is worse with eating, hunger or lying down/bending forward?

Yes

Dyspepsia (heartburn) likely

- Advise to stop NSAIDs (e.g. ibuprofen/aspirin), quit smoking ⇨ 141, limit alcohol, caffeine, spicy food, fizzy drinks, late night meals.
- If waist circumference > 80cm (woman) or 94cm (man), assess CVD risk ⇨ 127.
- Give **lansoprazole**⁵ 30mg daily for 14 days.
- Refer same week if any of: no better after 7 days treatment, symptoms return, painful/difficulty swallowing, persistent vomiting, blood in vomit or stool (occult blood positive), abdominal mass, weight loss, Hb < 12 (woman) or < 13 (man), new pain and > 50 years, or family history of stomach/oesophageal cancer.

No

- Assess for stress and anxiety ⇨ 86.
- If patient has a life limiting illness, consider giving palliative care ⇨ 170.
- Discuss/refer if:
 - Nausea/vomiting persists > 2 weeks.
 - Uncertain of diagnosis.

¹Common allergens include medication, food or insect bite/sting within the past few hours. ²Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ³Dilute 10mg morphine with 9mL of sodium chloride 0.9%. ⁴One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁵If HIV positive on atazanavir/ritonavir, avoid lansoprazole, discuss/refer.